

ESTABLISHED 2026 · PLAYA VISTA, CA

PLAYA VISTA **ASC** & REJUVENATION CLINIC

Cash-flow positive on the clinic alone — 10+ months before the first OR ever opens.

A vertically integrated surgical and wellness destination — where longevity programming accelerates surgical outcomes and cash-pay wellness funds the buildout.

\$43B+

U.S. ASC market

~67%

Blended margin at stabilization

~\$6.3M

Annual operating income
(Projected · Base Case)

THE PROBLEM

A Fragmented, Slow Standard of Care

6–12 wk

Surgical wait times patients endure for outpatient procedures.

0

Integrated luxury ASC + wellness facilities on the Westside today.

3+

Disconnected providers patients must navigate for surgery, recovery, and wellness.

Mission • Vision • Philosophy

Mission

Deliver world-class outpatient surgical care and evidence-based rejuvenation under one roof — premium healthcare made accessible, efficient, and transformative.

Vision

Become the leading vertically integrated ambulatory surgery and longevity destination on LA's Westside — known for clinical excellence, outcomes, and innovation.

Philosophy

Surgical outcomes improve when patients are optimized before procedures and supported with evidence-based recovery, regenerative, and longevity protocols afterward.

WHY NOW, WHY WESTSIDE

The Opening

Unserved Westside

No luxury ASC on the Westside integrates wellness into surgical care.

Dual-Stream Unlock

Insurance-backed surgical revenue and cash-pay wellness revenue, combined under one roof.

Early Cash Flow

Phase 1 clinic revenue starts 10+ months before OR activation — capital works while accreditation runs.

TARGET GEOGRAPHY

Playa Vista

Culver City

Marina del Rey

Santa Monica

Three Revenue Streams • One Facility

6,000–6,500 SF in Playa Vista. Phase 1 launches at Month 3–6; Phase 2 ORs activate at Month 16.

PHASE 1 • MO 3–6

Wellness + Aesthetics

\$419K/mo

73% margin • cash-pay • funds the OR build

PHASE 2 • MO 16

ASC Surgical Core

\$767K/mo

61% margin • insurance-backed

MO 16+

Combined Run-Rate

\$1.19M/mo

at stabilization

SERVICE CATEGORIES

6 Categories · 21 Revenue Lines

PHASE 1

Regenerative

PRP · Hormone/Peptide/GLP · IV Therapy (NAD+)

\$162.5K/mo

PHASE 1

Wellness & Aesthetics

Botox & Fillers · Laser · Morpheus8 · Emsculpt · CoolSculpting

\$166.8K/mo

PHASE 1

Recovery

Hyperbaric O₂ · Compression/NeuFit · Red Light

\$49.4K/mo

PHASE 1

Diagnostics

Advanced Biometrics · DNA · DUTCH · DEXA

\$26.5K/mo

PHASE 1

Aescape Robots

2× robotic massage stations · 85% margin · zero physician dependency

\$14K/mo

PHASE 2

Surgery

Pain Mgmt · Podiatry · Orthopedic · Spine (MIS)

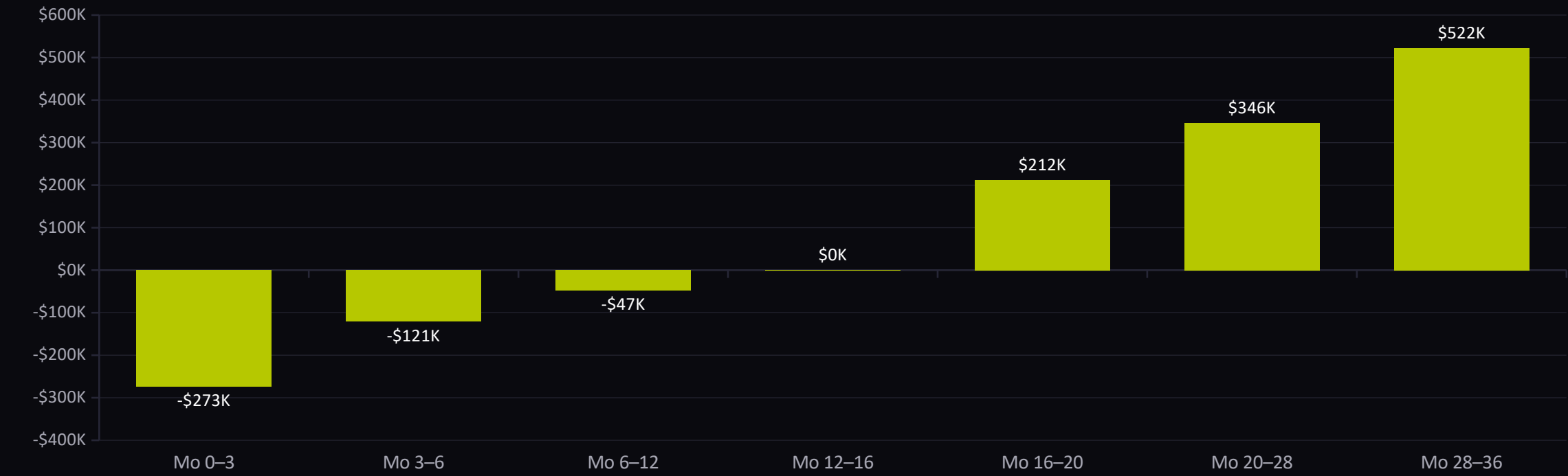
\$767K/mo

Profitable Before a Single Surgery

The de-risk. The clinic alone — no OR, no surgical license, no payer contracts — clears \$3.65M net annually.

Category	Monthly Gross	Monthly Net	Annual Gross	Annual Net
Regenerative	\$162,500	\$121,170	\$1,950,000	\$1,454,040
Aesthetics	\$166,775	\$114,214	\$2,001,300	\$1,370,574
Recovery	\$49,400	\$39,748	\$592,800	\$476,976
Diagnostics	\$26,500	\$17,405	\$318,000	\$208,860
Aescape Robots	\$14,000	\$11,900	\$168,000	\$142,800
PHASE 1 TOTAL	\$419,175	\$304,437	\$5,030,100	\$3,653,250

Path to Stabilization



Breakeven Month ~14 — before the second OR is licensed.

Unit Economics

Gross Revenue	\$1,186,175	100%
Variable Costs / COGS (~33%)	-\$391,438	
Monthly Net after COGS	\$794,737	67%
Rent (6,500 SF NNN)	-\$44,000	
Clinical Staff	-\$110,000	
Admin / Management	-\$28,000	
Marketing & CAC	-\$35,000	
Malpractice + Liability	-\$12,000	
Equipment Leases	-\$22,000	
Regulatory / Accreditation	-\$8,000	
Utilities / Software	-\$14,000	
Total Fixed Overhead	-\$273,000	23%
OPERATING INCOME / MO Projected - Base Case	\$521,737	44%

~67%

Net margin after COGS — before fixed overhead.

~\$6.26M

Annualized operating income at stabilization (base case).

SCENARIO BAND

Low • Base • Upside

Modeled operating-income outcomes across patient-volume and pricing assumptions.

LOW

\$1.07–1.57M

annual operating income

Gross/mo

\$371,780

Net/mo

\$247,562

Op income/mo

\$89–131K

EXPECTED

BASE • EXPECTED

~\$6.3M

annual operating income

Gross/mo

\$1,186,175

Net/mo

\$794,737

Op income/mo

~\$522K

UPSIDE

\$9.4–11.3M+

annual operating income

Gross/mo

\$1,588,600+

Net/mo

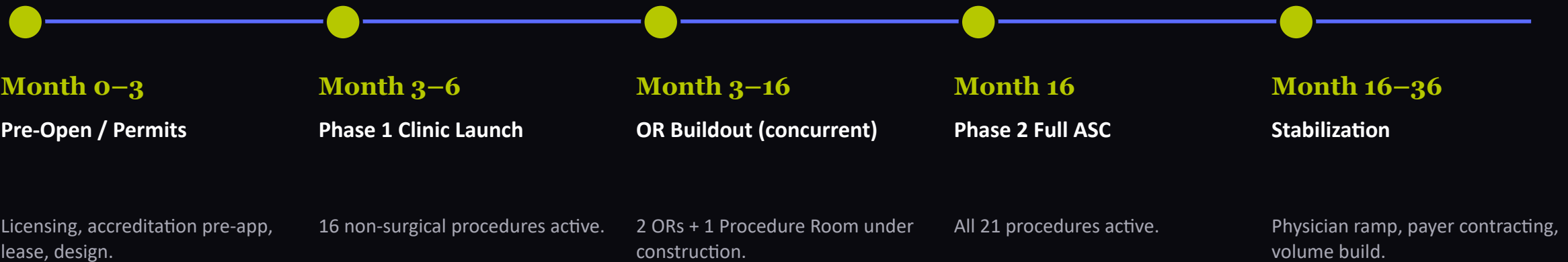
\$1,060,594+

Op income/mo

\$787–944K+

PHASED TIMELINE

From Permits to Full ASC



Class A • 6,500 SF • Playa Vista, CA

2 Operating Rooms

Surgical-grade, activated Month 16.

1 Procedure Room

Dedicated, activated Month 16.

Exam + Recovery

Exam rooms + post-op recovery bays.

2× Aescape Robots

Robotic massage stations (85% margin).

IV + Regenerative

IV therapy lounge + regenerative suite.

Med Spa Suite

Dedicated aesthetics & injectables suite.

2× Ammortal + HBOT

Recovery chambers + hyperbaric oxygen.

Support Spaces

Kitchen, restrooms, clinical/admin offices.

THE ASK

RAISING

\$5M

To open the Phase 1 clinic, build out two ORs and a procedure room, and carry working capital through Month 16 license activation.

INSTRUMENT	VALUATION CAP	FIRST REVENUE
SAFE · milestone-tranched	Targeting \$15M–\$25M	Month 3–6 (clinic)

Converts at the next priced round. Cap and discount finalized with lead investor.

USE OF FUNDS

Phase 1 Clinic Buildout	\$500K–\$900K
Phase 2 OR Construction + Equipment	\$2.5M–\$3.25M
Aesthetic & Recovery Equipment	\$400K–\$800K
Working Capital (to Month 16)	\$500K–\$750K
Pre-Opening (legal, licensing)	\$500K–\$600K
DEPLOYABLE RANGE	\$4.4M–\$6.3M

Priced on Proof, Not Promise

A SAFE released in two milestone-tied tranches — capital deploys against demonstrated progress, and the larger tranche prices after the clinic proves the model.

Tranche 1 — Clinic & Pre-Opening

Funds the Phase 1 buildout, accreditation, and working capital. The earliest, highest-risk capital — converts at the cap.

Tranche 2 — OR Construction

Releases at the clinic-open milestone, once revenue is flowing. De-risked capital funding the Phase 2 surgical core.

Physician Co-Ownership Pool

A defined equity pool reserved up front for surgeon partners under California ASC rules — aligning surgical volume without surprise dilution.

Investor Protections

Valuation cap, conversion discount, and pro-rata rights for the lead — standard early-stage terms, calibrated in the term sheet.

Financing Layers

Equipment leases, NMTC, and SBA 7(a) considered for OR buildout — reducing the equity required inside the \$5M.

Final structure subject to lead-investor term sheet and counsel. Not an offer to sell securities.

This Is a Platform Bet

Playa Vista is the proof site. The model is built to replicate — same clinic-first sequencing, same payer mix, same wellness flywheel — across affluent metro pockets where Westside dynamics translate.

01 · GEOGRAPHIC REPLICATION

Westside Playbook, Other Metro Pockets

Demand profile maps to Newport Beach, Manhattan Beach, Scottsdale, Austin, Miami. Lower buildout risk on site #2 onward.

02 · CLINIC-FIRST SEQUENCING

Each Site Cash-Flows Before It Cuts

Clinic opens Month 3–6, generating \$300–400K/mo to fund the OR buildout. Capital efficiency compounds per location.

03 · RECURRING REVENUE LAYER

Membership & Programs

Longevity, hormone, GLP-1, and IV/peptide therapy support subscription structures — episodic visits become retained patients.

04 · PROPRIETARY TECH MOAT

Aescape, Ammortal, HBOT

Robotic massage, recovery chambers, and HBOT at 80–85% margins with near-zero physician dependency. Margin defense at scale.

05 · SURGICAL-WELLNESS LOOP

Pre-Op Optimization → Recovery

Wellness drives the surgical pipeline; surgical patients flow back into wellness. The integrated loop is the network effect.

06 · STRATEGIC OPTIONALITY

Exit Routes Open at Each Stage

Multi-site footprint opens hospital systems, PE-backed ASC roll-ups, and longevity platforms — optionality preserved throughout.

Who Builds This — and What's Next

Founder / Operator — Laura Brody	IN PLACE
Medical Director (MD/DO)	RECRUITING
Wellness Clinical Director	RECRUITING
Aesthetics / Injector (NP/PA)	RECRUITING
Chief AI Officer	RECRUITING
Surgeon Partners (Ortho/Spine)	RECRUITING
Anesthesia (MD vs. CRNA)	TBD

THE PATH FORWARD

- 01 Finalize raise structure, close commitments.
- 02 Execute Playa Vista Class A lease.
- 03 Begin Phase 1 clinic buildout (Mo 3–6 open).
- 04 File AAAHC pre-app + California SURGC.
- 05 Procure Aescape, Ammortal, HBOT, lasers.
- 06 Hire clinical + medical directors, injector.
- 07 Commence OR construction (Mo 16 target).
- 08 Onboard surgeons + execute payer contracts.

LET'S BUILD THE WESTSIDE'S FIRST INTEGRATED ASC

Vertically integrated surgical care and longevity — under one roof, in Playa Vista.

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